



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 2220/08

BEFORE:

B. Kalvin : Vice-Chair
V. Phillips: Member Representative of Employers
M. Ferrari: Member Representative of Workers

HEARING:

October 29, 2010 at Toronto
Oral

DATE OF DECISION:

December 1, 2010

NEUTRAL CITATION:

2010 ONWSIAT 2744

DECISION(S) UNDER APPEAL: WSIB ARO decision dated February 26, 2007

APPEARANCES:

For the worker: S. M. Philbert, lawyer

For the employer: P. Harris, paralegal

Interpreter: A. Kowalczyk, Polish

**Workplace Safety and Insurance
Appeals Tribunal**

505 University Avenue 7th Floor
Toronto ON M5G 2P2

**Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail**

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REASONS

- [1] These are the reasons for decision of the Workplace Safety and Insurance Appeals Tribunal with respect to an appeal by a worker from a decision of the Workplace Safety and Insurance Board (the “Board”) concerning the worker’s entitlement to benefits following a workplace accident.
- (i) **Background**
- [2] The background to this appeal is as follows. The worker is 64 years old. In 1979, he began working for a manufacturing company as a production engineer. The worker’s job entailed writing and monitoring computer programs for production machinery.
- [3] On December 12, 2000, the worker slipped and fell in a snowy corridor upon entering the employer’s plant. His head and elbow hit the ground. He was dazed and rested for a while on a bench before returning to his work station. On the following day, he reported the injury to the employer’s medical office. He also went to see his family physician, Dr. A. Pinto, who filled out a *Physician’s First Report* and sent it to the Board. In that report, Dr. Pinto stated that the worker slipped on snow, fell backwards and struck his “occiput and L elbow.” Dr. Pinto diagnosed contusions and a neck strain.
- [4] The worker did not lose any time from work immediately following the accident. On January 5, 2001, he was assessed by a physiotherapist who reported that as a result of the accident the worker was complaining of decreased range of motion in the neck, nausea, dizziness, headaches, and tenderness in the upper cervical spine. The physiotherapist diagnosed a “cervical whiplash grade 2.”
- [5] The worker continued to work from the date of the accident until December 8, 2001. The reason that the worker stopped working on that date is a matter of dispute. It appears that the employer reported to the Board that the worker was released on that date as part of a general restructuring of its workforce. At the hearing of this appeal, the worker testified that he was fired because of poor work. The worker has not returned to work since December 8, 2001.
- [6] Some time after he stopped working - it is not clear exactly when – the worker initiated a claim for compensation benefits in relation to his fall of December 12, 2000. The worker claimed that the accident caused him to sustain a head injury which in turn resulted in a psychotraumatic disability. The worker’s claim was denied by a Board Claims Adjudicator. The worker objected to the Claims Adjudicator’s decision, and his objection was considered by an Appeals Resolution Officer (“ARO”) in the Board’s internal Appeals Branch. In a decision dated February 26, 2007, the ARO denied the worker’s objection and confirmed the denial of benefits for a psychotraumatic disability. In coming to that decision, the ARO stated that there was no evidence that the worker actually hit his head at the time of the accident. The ARO noted further, that the worker had a pre-existing history of headaches and psychological problems. The ARO referred to and relied on medical reports which concluded that the worker did not sustain a brain injury at the time of his accident, and his psychological problems were likely due to having been fired, rather than from injuries sustained in the accident. Despite the worker’s claim that in the year following the accident he was only able to perform a modified version of his job, the

ARO concluded that the worker performed his regular duties from the date of the accident until he stopped working a year later. The ARO concluded as follows:

The evidence clearly supports that the worker's current complaints are the result of psychological problems that have long predated his compensable injury. There is extremely little evidence of any neurological or psychological impact from his compensable injury. Any injuries arising from the accident resolved after his short period of physiotherapy in January 2001.

[7] The worker now appeals the ARO decision to this Tribunal.

(ii) Issues

[8] The issues to be resolved on this appeal are as follows:

1. Is the worker entitled to benefits for a psychotraumatic disability?
2. If so, is the worker entitled to a permanent impairment for a psychotraumatic disability?
3. Is the worker entitled to loss-of-earnings benefits?

(iii) Analysis

Is the worker entitled to benefits for a psychotraumatic disability?

[9] Because the worker's accident occurred in 2000, his entitlement to benefits is governed by the *Workplace Safety and Insurance Act, 1997* (the "WSIA"). Section 126 of the WSIA requires this Tribunal to apply Board policy. The Board's policy on *Psychotraumatic Disability* is set out in *Operational Policy Manual* Document #15-04-02. It reads as follows:

Policy

A worker is entitled to benefits when disability/impairment results from a work-related personal injury by accident. Disability/impairment includes both physical and emotional disability/impairment.

Guidelines

General rule

If it is evident that a diagnosis of a psychotraumatic disability/impairment is attributable to a work-related injury or a condition resulting from a work-related injury, entitlement is granted providing the psychotraumatic disability/impairment became manifest within 5 years of the injury, or within 5 years of the last surgical procedure.

Psychotraumatic disability/impairment is considered to be a temporary condition. Only in exceptional circumstances is this type of disability/impairment accepted as a permanent condition.

Psychotraumatic disability/impairment resulting from organic brain damage is assessed as a permanent disability/impairment.

Psychotraumatic disability entitlement

Entitlement for psychotraumatic disability may be established when the following circumstances exist or develop

- Organic brain injury secondary to
 - traumatic head injury
 - toxic chemical including gases
 - hypoxic conditions, or
 - conditions related to decompression sickness
- As an indirect result of a physical injury
 - emotional reaction to the accident or injury
 - severe physical disability/impairment
 - reaction to the treatment process
- The psychotraumatic disability is shown to be related to the extended disablement and to non medical, socioeconomic factors, the majority of which can be directly and clearly related to the work-related injury

[10] There is no question in this case that prior to the accident of December 12, 2000, the worker suffered from pre-existing headaches as well as psychological problems. These are documented in reports from his treating physicians. In a report dated March 8, 2002, the worker's family physician, Dr. J. Kowalewski stated the following:

Early and middle 1999, tension headaches and Depressive states complaints of excessive stress at work poor memory, inability to concentrate – treated with Prozac 20 mg daily, 30 tablets [sic].

[11] In a letter dated December 12, 2001, that is, four days after the worker stopped working, Dr. M. D. Levitan, a neurologist reported as follows:

Thank you for asking me to see this gentleman once again. Please refer to my consultation report few of 1999 [sic]. He reported tension type headaches back then and his examination and CT scan of the head were normal [sic]. These headaches eventually improved significantly according to his report of today. Unfortunately he suffered a closed head injury last December in a slip and fall at his building where he has worked as a computer programmer. Apparently it was a snowy entrance [sic]. He fell on the back of his head and was stunned.

[12] These reports clearly document that the worker suffered from headaches, stress, and depression prior to his slip and fall at work on December 12, 2000. However, pre-existing condition does not automatically disentitle a worker to benefits. According to the Board's policy entitled Aggravation Basis, set out in *Operational Policy Manual* Document No. 11-01-15, a worker is entitled to benefits if a workplace accident aggravates a pre-accident impairment. The policy states:

Policy

In cases where the worker has a **pre-accident impairment** and suffers a minor work-related injury or illness to the same body part or system, the WSIB considers entitlement to benefits on an **aggravation basis**.

Generally, entitlement is considered for the acute episode only and benefits continue until the worker returns to the **pre-accident state**.

[13] Having considered the evidence and submission put forward at the hearing of this appeal, we find that the worker is entitled to benefits for a psychotraumatic disability. Our reasons for this conclusion are as follows.

[14] First, and with respect to the ARO who found otherwise, we accept that the worker sustained an injury to his head when he slipped and fell at work on December 12, 2000. There is clear and contemporaneous documentation of such an injury. The *Physician's First Report* signed by Dr. Pinto on December 13, 2000, that is, the day after the accident, states that the worker fell "striking occiput and L elbow." Similarly, the notes recorded on December 13, 2000 by staff at the employer's internal medical centre state that the worker fell and struck his head and left elbow. Thus, contrary to the conclusion reached by the ARO, we find that the worker did sustain a head injury on December 12, 2000.

[15] Second, in our view, a preponderance of the medical evidence suggests that the worker's head injury on December 12, 2000 resulted in an exacerbation of pre-existing headaches and other psychological symptoms. We acknowledge that the medical evidence on this question is not uniform, and that some of that evidence indicates that the accident of December 12, 2000 was not a factor which contributed significantly to the worker's psychological impairment. This evidence is referred to and relied on by the ARO. For instance, the worker was assessed over the course of five days in November 2002 at the Toronto Rehabilitation Institute ("TRI"). In their report, the TRI assessors indicate that while it is difficult to determine whether the worker's cognitive deficits and mood disorder are attributable to the accident or to other factors, they conclude that the worker's symptomatology is "probably not" the result of a brain injury sustained in the fall. The report reads as follows:

With regard to his emotional functioning, [the worker] is definitely reporting increased emotional distress, including severe anxiety and moderate to severe depression. Clearly, anxiety and depression are playing a role in [the worker's] current level of cognitive functioning and at this time it is difficult to determine the extent to which his cognitive deficits are the result of the accident versus heightened emotional distress. However, two things need to be emphasized: First [the worker] did return to work for approximately one year after he was injured; he reportedly decompensated after the company was restructured, resulting in his termination. Second, the presence of focal cognitive deficits following a very mild concussive head injury two years post injury is highly atypical and is probably not, in our opinion, due to the residual effects of a mild brain injury. On the other hand, it is reasonable to expect a degree of cognitive impairment related to mood disorder, and in our view, his current test results are likely reflective of this.

[16] The opinion of the TRI assessors was endorsed by two Board Medical Consultants. In an opinion dated February 19, 2003, Dr. Scullion wrote the following:

The psychometric testing by the WSIB head injury clinic 10/1/03/ gives the opinion that the testing shows focal cognitive deficits which would be highly atypical of residual effects from a mild brain injury.

...

Due to the reasons given above and the opinion expressed by the WSIB head injury psychometric testing – there is no evidence of a mild brain injury from the fall of 12/12/00.

Similarly, in a report dated December 29, 2006, Dr. J. Dudley states:

The worker's accident occurred in December 2000. He was laid off from work approximately one year later. Medical reporting suggested his symptoms had resolved in terms of headaches and nausea shortly after the accident. After his layoff he was described as being depressed and attributes his lay off to poor work performance secondary to the previous head injury. There is no evidence of medical attention for the year prior to his lay off. The reporting of symptoms, in my mind, seem more in response to the actual lay off rather than to any ongoing symptoms attributable to the December 2000 head injury. This is clearly stated in the comprehensive assessment report of the Toronto Rehab Institute in January 2003.

- [17] However, in contrast to the evidence set out above, there is also a substantial body of medical evidence which indicates that there is a causal relationship between the workplace accident of December 12, 2000 and the onset, or exacerbation of cognitive and emotional problems. For instance, in a report dated May 5, 2003, Dr. Z. R. Mech, a psychiatrist, states:

Clinically he suffers from a Major Depressive Disorder, which has originated or has been perhaps aggravated by work injury in December of 2000 [sic]. His depression is bound inexorably to his intellectual functioning now.

...

In my opinion he has clearly suffered brain damage as a result of his fall.

- [18] In a subsequent report dated November 20, 2006, Dr. Mech wrote the following:

From the time of the accident he suffered at base of neck, shoulders and back as well as headaches, which became progressively more severe and incapacitating and memory problems characteristic of post concussional syndrome [sic].

Dr. Mech diagnosed the worker with a "Major Depressive Disorder secondary to work injury," and stated that the worker's "physical symptoms and depressive problems evolved and were a direct progression originating in the accident."

- [19] As noted earlier, the worker was treated by Dr. Levitan, prior to and after the workplace accident. Dr. Levitan first treated the worker in 1999, and then again in December 2001. In a letter dated March 4, 2008, Dr. Levitan states that the tension headaches for which he first treated the worker in 1999 were "exacerbated following a fall at work a year later."

- [20] The Case Record contains many reports written by Dr. D. Ouchterlony, a physician practising in the field of neurorehabilitation. Dr. Ouchterlony opines repeatedly that the worker's accident of December 12, 2000 resulted in a head injury which affected the worker's cognitive and emotional functioning. For instance, in report dated July 17, 2004, Dr. Ouchterlony states that the worker's accident "impaired his ability to function normally at work." And further:

[The worker] suffered a closed head injury as a result of the accident of December 12 as evidenced by the strength of the initial injury, the ongoing clinical course and the diagnosis of his treating team. Dr. Kowalski, Dr. Vistek, Dr. Mech and Dr. Levitan and myself all felt that he sustained a head injury. It is not out of keeping that he was able to return to work for one year, as he was supported at work and indeed was not functioning at a competitive level during that year. As a result of his poor performance he was let go in a downsizing.

In a report dated April 20, 2006, Dr. Ouchterlony states that the worker “definitely has an ongoing head injury with continuing problems, and I would suggest that he has some depression.” There are many other reports from Dr. Ouchterlony to a similar effect.

[21] As noted above, acknowledging that the medical evidence is not uniform, we find on a preponderance of that evidence that when he fell at work on December 12, 2000, the worker sustained a head injury which resulted in cognitive problems and aggravated pre-existing emotional problems. We prefer the evidence that supports a causal link between the accident and the ensuing cognitive and emotional difficulties for the following reasons.

[22] First, the main medical report casting doubt on a causal link between the worker’s accident and his subsequent cognitive and emotional problems is the TRI report quoted earlier. As noted above, both Drs. Scullion and Dudley relied on that TRI report in coming to their conclusions. However, the TRI report quoted above was not the only medical report prepared at the TRI when the worker was assessed there. The report quoted above was authored by Dr. P. Comper, a clinical psychologist and J. Vernon, a psychometrist. Another report emanating from the TRI is authored by Dr. C. Whitehead and dated February 13, 2003. In this report, Dr. Whitehead states that the worker was assessed in relation to a brain injury which he sustained on December 12, 2000. Dr. Whitehead states that the worker was seen for a full-day assessment by Dr. Cullen and additionally for cognitive assessment. Dr. Whitehead provides a discharge diagnosis of a brain injury and mood disorder relating to the worker’s accident of December 12, 2000. Thus, the reports from the TRI are not uniform on the question of whether a causal link exists between the worker’s accident and his cognitive and emotional problems.

[23] Second, a major factor accounting for the bifurcation of medical opinion pertains to the underlying facts of the worker’s situation. The medical opinions which cast doubt on a causal link between the worker’s accident and his cognitive and emotional difficulties note that after the accident, the worker was able to return to work for a year following the accident. The medical opinions supporting a causal link, particularly those of Drs. Mech and Ouchterlony, rely on a history in which the worker was unable to cope with his work duties after the accident, and accordingly, due to poor performance, lost his job.

[24] In our view, the evidence presented at the hearing of this appeal supports the worker’s claim that although he worked for a year following his slip and fall at work in December 2000, he had serious cognitive difficulties which rendered him unable to cope with his regular tasks. The worker testified that during that year he performed only routine assignments and the more complex aspects of his job were taken over by a co-worker KV. KV testified at the hearing of this appeal and confirmed the worker’s testimony on this point. KV testified that prior to the worker’s slip and fall the worker was coping with his regular programming duties. KV testified that after the accident there was a noticeable downturn in the worker’s quality of work, and in particular, numerous mistakes in his programming became apparent. KV testified that he often had to correct errors made by the worker and that this became a consistent pattern. KV testified that he and the worker eventually worked out an arrangement whereby the worker would take over the routine less complex tasks while KV would perform all the more complex aspects of the programming job which the worker appeared no longer capable of performing adequately.

[25] Both the worker’s and KV’s testimony was confirmed by another witness ZM. ZM testified that he worked at the employer as a machine operator. ZM testified that when problems

arose with a machine, the operator would often have to seek the assistance of a programmer to fix the problem. ZM testified that prior to the worker's accident he was always able to seek and obtain assistance from the worker. After the worker's accident, ZM testified that the worker became unwilling to provide assistance and told ZM he was having difficulty coping with his work. Thus, the evidence of all three witnesses at the hearing of this appeal was consistent on this point, namely, that after his accident in December 2000, there was a noticeable decline in the worker's ability to cope with the complex aspects of his programming duties and there was a noticeable decline in the quality of his work. Accordingly, based on this evidence, we find that the worker's accident significantly impaired his ability to perform the work he had done prior to the accident. For this reason, we place greater reliance on the medical opinions which are consistent with this history than on the medical opinions which are premised on a history that the worker was able to return to work for a year following his accident. While the worker did return to work, the evidence indicates, in our view, that his ability to perform his regular job duties was immediately compromised following the head injury which he sustained on December 12, 2000.

- [26] For the reasons set out above, we find it more likely than not that when he slipped and fell at work on December 12, 2000, the worker sustained a head injury which caused cognitive difficulties, which in turn aggravated pre-existing emotional problems. We find that the worker is entitled to benefits for a psychotraumatic disability.

Is the worker entitled to a permanent impairment for a psychotraumatic disability?

- [27] The WSIA defines "impairment" and "permanent impairment" as follows:

"impairment" means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss;

"permanent impairment" means impairment that continues to exist after the worker reaches maximum medical recovery.

- [28] In our view, the psychotraumatic disability which resulted from the worker's compensable accident of December 12, 2000 constitutes a permanent impairment. The evidence in this case leads us to conclude that the accident aggravated the worker's pre-existing psychological condition on a permanent basis. The evidence reveals that the worker continued to suffer from cognitive and emotional problems for many years following the compensable accident of December 12, 2000.

- [29] It is important to note however, that where a worker sustains a permanent impairment to a body part which was previously impaired, the worker's resulting non-economic loss ("NEL") benefit will compensate not for the entire impairment, but only for the portion of the impairment which is made worse by the compensable accident. The Board's policy entitled *Effect of a Pre-existing Impairment* set out in *Operational Policy Manual* Document No. 18-05-05 states:

Pre-existing non-work-related impairments

New injury affecting a different area of the body

If a worker has a pre-existing permanent impairment that is not work-related, and a new work-related permanent impairment to a different area of the body, the WSIB rates the work-related impairment on its own.

New injury affecting the same body area

If both impairments affect the same area of the body, and the pre-existing impairment is measurable, the WSIB

- rates the total impairment to the area
- determines the rating for the pre-existing impairment, and
- subtracts the rating for the pre-existing impairment from the total impairment rating to get the rating for the new work-related impairment.

If the pre-existing impairment is not measurable, the WSIB

- rates the total area's impairment, and
- reduces this rating according to the significance of the pre-existing impairment (see pre-accident disability in 14-05-03, Second Injury and Enhancement Fund).
- if minor, there is no reduction
- if moderate, there is a 25% reduction
- if major, there is a 50% reduction.

NOTE

A pre-existing impairment is "measurable" or non-measurable" depending on whether it can be rated using the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 3rd edition (revised). This determination is based strictly on the clinical information available at the time of the work-related injury.

[30] In this case the worker has claimed entitlement to benefits for a psychotraumatic disability. For reasons set out earlier, we find that the worker suffered from a pre-existing psychotraumatic disability which was permanently aggravated by his compensable workplace accident. The worker's compensable impairment is therefore an impairment to "the same area of the body" for the purposes of the Board's policy. Further, in our view, the worker's pre-existing psychological impairment was a measurable condition. It was a condition which had been identified and treated in the year prior to the worker's compensable accident. We therefore find that the worker's pre-existing impairment must be rated and subtracted from his total impairment in order to properly determine the quantum of the worker's compensable permanent impairment for his psychotraumatic disability.

Is the worker entitled to loss-of-earnings benefits?

[31] For reasons set out earlier we find that the worker's compensable accident aggravated a pre-existing psychotraumatic disability and thereby rendered the worker unable to cope with the requirement of his job. We accept the worker's testimony that he was fired on December 8, 2001, because of poor job performance that was caused by his compensable accident. We therefore find that the worker suffered a loss of earnings which is attributable to his compensable accident, and is therefore entitled to loss-of-earnings benefits as of December 9, 2001.

DISPOSITION

[32] The appeal is allowed:

1. The worker is entitled to benefits for a psychotraumatic disability.
2. The worker is entitled to a permanent impairment award for his psychotraumatic disability. The worker suffered from a measurable pre-existing psychotraumatic impairment which must be rated and subtracted from his total impairment in order to determine the quantum of his compensable permanent impairment. The quantum of the worker's compensable permanent impairment is to be determined by the Board subject to the usual rights of appeal.
3. The worker is entitled to loss-of-earnings benefits from December 9, 2001. The quantum and duration of benefits is to be determined by the Board subject to the usual rights of appeal.

DATED: December 1, 2010

SIGNED: B. Kalvin, V. Phillips, M. Ferrari